Session 2 (FIND & READ): Solutions and facilitation notes

Foundations of Clinical Research · Level 1 · Neudata · *Instructor copy. Do not share before the session.*

Model answers. Other answers are fine if they are specific and consistent.

Recap quiz: Session 1 answers

1. **Paper 1 (Boulet 2024) is a trial** (pilot RCT); **Paper 2 (Hyun 2023) is a cohort** (matched registry cohort).
2. **P** adults with sepsis · **E** positive cumulative fluid balance by day 3 · **C** neutral or negative balance · **O** death · **T** by day 28 (a PECO question: E = Exposure).
3. **Feasible**: a mortality outcome needs more than 1,000 patients.
4. **"To estimate the proportion fluid-positive on day 3"**: a measurable verb. "Understand" is not measurable.
5. **H0:** cumulative fluid balance at day 5 is the same with the restrictive protocol and with usual care.

If many miss 4 or 5, spend one extra minute: objectives use measurable verbs; H0 always states "no difference".

Check your understanding (segment 2.1): AND, OR or NOT?

1. **AND** (septic shock AND fluid restriction). 2. **OR** (restrictive OR conservative). 3. **Fewer** (AND narrows). 4. It removes every paper that mentions children anywhere, including good adult studies that mention children once.

Check your understanding (segment 2.2): OK or not OK?

1. **Not OK**: patchwriting is plagiarism even with a citation. 2. **OK** (but rare in medical writing). 3. **Not OK**: someone else's finding needs a citation. 4. **Not OK**: you may repeat an error, so read and cite the original.

Check your understanding (segment 2.3): spot the spin

1. Causal word for an observational design. Rewrite as *"a positive fluid balance was associated with higher mortality"*.
2. p = 0.31 is no evidence of a difference. Rewrite as *"no significant difference; the confidence interval was wide"*.
3. Headline is a secondary outcome. Lead with the primary result instead.
4. Before–after change without a comparison group. Rewrite as *"mortality was lower after implementation; without a control group we cannot attribute this to the protocol"*.

Exercise 1: Boolean search builder

Warm-up: `septic shock OR fluids` gives more hits (either term). `sepsis` gives more hits than `sepsis NOT neonatal` (NOT removes papers).

Model string for the running example (as in the demo):

```
("Shock, Septic"[Mesh] OR "septic shock"[tiab] OR sepsis[tiab])
AND ("Fluid Therapy"[Mesh] OR "fluid restrict*"[tiab] OR "restrictive fluid*"[tiab]
  OR "conservative fluid*"[tiab])
AND ("Mortality"[Mesh] OR mortality[tiab] OR death[tiab])
```

Sensible extra synonyms for the I block: `de-resuscitation[tiab]`, `deresuscitation[tiab]`, `"fluid balance"[tiab]`, `"Water-Electrolyte Balance"[Mesh]`.

Sensible filters (each with a reason): article type *Randomized Controlled Trial* + *Systematic Review* (treatment question); *Adult: 19+ years* (our population); publication date *10 years* (current practice). Avoid *Free full text* (loses key papers).

Model string for Scenario C (delirium)

```
(Delirium[Mesh] OR delirium[tiab] OR "acute confusion"[tiab])
AND ("Hip Fractures"[Mesh] OR "hip fracture*"[tiab])
AND (prevent*[tiab] OR "multicomponent"[tiab] OR "non-pharmacological"[tiab] OR
  bundle[tiab])
```

Common faults

- Missing brackets: `A OR B AND C` is read differently from `(A OR B) AND C`.
- AND inside a block (`septic shock AND sepsis`). Synonyms need OR.
- A block for the comparison ("usual care"). It adds nothing and loses papers.
- `NOT` used to remove a topic the paper only mentions in passing.
- Search words in the local language: PubMed indexes English titles and abstracts.

Exercise 2: Paraphrasing (model answers)

Part A

Version	Verdict	Why
a)	**Problem: patchwriting**	Same structure and almost the same words as the source; one word swapped ("mortality" for "death"). Plagiarism even though it is cited.
b)	**OK**	New structure, adds context (matched registry cohort, 28-day mortality), keeps the calibrated word "associated", gives numbers and the citation.
c)	**Problem: distorts the source**	"Giving more fluid ... doubles the risk" turns an observational *association* into causation, and "doubles" overstates (HR 2.13 only for day 2; 1.56 for day 3). Citation does not fix misrepresentation.

Version	Verdict	Why
d)	**Problem: no citation**	Good wording, but presenting someone else's finding without a citation is plagiarism.

Part B: model paraphrase

"Mortality and days alive without organ support at day 28 did not differ between the two groups, although the pilot trial was too small to detect such differences [1]."

What good looks like: new structure; facts unchanged (28 days, mortality, days alive without life support, similar); no stronger claim (do **not** write "restriction is safe"); citation. Adding "the trial was too small" is accurate context from the paper (48 patients; not powered for mortality).

Exercise 3: Paper 1's abstract (answer key)

Item	Answer (from the abstract)
Design (and blinding)	Multicentre, randomised, controlled **pilot** trial; single-blind (patients and statisticians blinded, clinicians not)
P	Adults 18–85 years with septic shock, in the first 24 h of vasopressor infusion, in three ICUs; excluded: AKI needing renal replacement, end-stage CKD, severe malnutrition
I vs C	Optimised fluid restriction (reducing maintenance fluids and fluids for drug dilution during the first 7 days) vs standard fluid strategy
Primary outcome	**Cumulative fluid balance (ml/kg) in the first 5 days**, not mortality
Numbers	1,201 screened; 50 randomised; 2 withdrew (control); 48 analysed (24 per group)
Main result	Median cumulative balance at day 5: 6.9 vs 35.0 ml/kg; absolute difference 13.2 ml/kg (95% CI −15.2 to 41.6), p = 0.42. The 13.2 is a difference in means (the Methods report the mean difference with its 95% CI), so it does not equal the gap between the medians.
Conclusion	"...did not reduce the overall fluid balance at day 5"
Matches the result?	**Yes**. The CI includes zero, and the conclusion says "did not reduce".
Spin in the abstract?	**No** in the abstract conclusion. (Spin appears in the *Discussion*, which calls p = 0.42 a "non-significant trend".) Watch-point: the fluid *volumes* (89.7 vs 114.3 ml/kg, p = 0.0506) are a secondary result, and the abstract reports four decimals for a borderline p. This is mild emphasis, not a claim.
Registration	ClinicalTrials.gov **NCT04947904** (registered 1 July 2021, before recruitment began in September 2021)
Answers our 28-day mortality question?	**No.** It is a small pilot powered for fluid balance. 28-day mortality was a secondary outcome and was similar between groups: "no evidence of a difference", not "no difference".

Fast finishers (Paper 2): the title says **"Impact"** (causal); the abstract conclusion correctly says **"associated"**.

Common errors to correct in the debrief

- Writing "mortality" as the primary outcome.
- Copying only the p-value and not the estimate with its 95% CI.
- Calling the trial "double-blind". Clinicians, who prescribe the fluids, were not blinded.
- Reading the medians (6.9 vs 35.0) as "five times less fluid balance, so it works". The CI crosses zero.

Exercise 4: Proposal builder 2 (what good looks like)

The trainer models this live for the running example (the feasible pilot version A from Session 1); each participant then does the same for their own question. Use it as the quality benchmark, not as the answer.

Search (documented): PubMed, date, the string from Exercise 1, filters (article type RCT + systematic review; adults; 10 years), number of hits; plus a check of ClinicalTrials.gov for ongoing trials.

Working title: *Restrictive versus usual fluids in adults with septic shock: a pilot randomised trial* (13 words).

Keywords: Fluid Therapy; Water-Electrolyte Balance; Critical Care; Vasoconstrictor Agents (MeSH terms not already in the title).

Problem statement (≈ 300 words, three paragraphs)

1. *Known:* septic shock is a common cause of ICU death. Early fluid resuscitation restores perfusion, but continued fluid often produces a positive fluid balance, which observational studies associate with higher mortality (e.g. Hyun 2023).
2. *Not known:* large trials of restrictive fluids (CLASSIC 2022; CLOVERS 2023) found no mortality difference, and a pilot targeting hidden "non-resuscitative" fluids (Boulet 2024) did not significantly reduce fluid balance at day 5. Whether a restrictive protocol reliably reduces fluid balance is still unclear.
3. *Question and objectives:* in adults with septic shock, does a restrictive fluid protocol, compared with usual care, reduce cumulative fluid balance at day 5? Primary objective: to compare day-5 fluid balance between the two groups.

Science rule: the Introduction never contains your own data or results (no "our audit"). They belong in the Results. A local figure may appear only if it comes from a publication, with its reference.

Five key references (model)

#	Reference	Why included
1	Boulet N, et al. *Crit Care* 2024;28:429 (pilot RCT, n = 48)	Closest design and outcome; primary outcome not significantly different
2	Hyun DG, et al. *Ann Intensive Care* 2023;13:105 (matched cohort)	Positive balance on days 2–3 associated with higher 28-day mortality

#	Reference	Why included
3	Meyhoff TS, et al. (CLASSIC) *N Engl J Med* 2022;386:2459–70	Large RCT, septic shock, no difference in 90-day mortality
4	Shapiro NI, et al. (CLOVERS) *N Engl J Med* 2023;388:499–510	Large RCT, sepsis-induced hypotension, restrictive vs liberal, no difference in 90-day death before discharge
5	Evans L, et al. Surviving Sepsis Campaign guidelines 2021. *Crit Care Med* 2021;49:e1063–e1143	Current guideline; shows the remaining uncertainty

Instructor note: check citation details against PubMed before distributing. Participants must verify every reference themselves, which is exactly the habit to practise.

The live part (minutes 0–10): what the trainer shows

1. Run the full string from the PubMed demo (`s2_demo.md`, Strings to copy, Step 3) with the filters RCT and systematic review, adults, 10 years. Write the search record on screen: database, date, string, filters, number of results. The number changes every day, so read it from the screen.
2. Screen the first titles aloud. Keep adults with sepsis or septic shock, a comparison of fluid strategies or fluid balance, and a trial, cohort or review. Skip children, burns and surgery, and trials of one fluid type against another (crystalloid against colloid). Participants suggest papers and keywords in the chat.
3. Save Boulet 2024 and CLASSIC to Zotero (*Send to* → *Citation manager*) and fill their two worksheet rows together (model rows below).
4. Search ClinicalTrials.gov for "septic shock" and "fluid" to show how to look for ongoing trials.
5. Ask for keywords in the chat. Model answer: the four MeSH terms above.

Literature review worksheet: model rows

#	Study	Design, size	Compared	Primary outcome: result	Key limitation	Why it matters for our question
1	Boulet 2024, *Crit Care*	Pilot RCT, 3 ICUs, 50 randomised, 48 analysed	Restricting maintenance and drug-dilution fluids for 7 days vs standard care	Cumulative fluid balance at day 5: median 6.9 vs 35.0 ml/kg, p = 0.42, no significant difference	Small, not powered for mortality, clinicians not blinded	Closest design and outcome to our pilot
2	Hyun 2023, *Ann Intensive Care*	Propensity-matched cohort, multicentre registry, 20 hospitals, 2,516 patients	Positive vs negative cumulative fluid balance at the end of ICU day 1, 2 or 3	28-day death: HR 1.17 (95% CI 0.85 to 1.60) at day 1, 2.13 (1.48 to 3.06) at day 2, 1.56 (1.10 to 2.22) at day 3	Observational, confounding possible	Fluid balance is linked with death, so it is worth reducing
3	Meyhoff 2022, *N Engl J Med* (CLASSIC)	RCT, 1,554 adults with septic shock	Restrictive vs standard intravenous fluid	90-day death 42.3% vs 42.1%, no difference	Clinicians not blinded	Restriction did not change mortality in a large trial

#	Study	Design, size	Compared	Primary outcome: result	Key limitation	Why it matters for our question
4	Shapiro 2023, *N Engl J Med* (CLOVERS)	RCT, 1,563 adults with sepsis-induced hypotension	Restrictive (earlier vasopressors) vs liberal fluid	Death before discharge home by day 90: 14.0% vs 14.9%, no difference	Open label, early phase of care only	Confirms CLASSIC in a different population
5	Evans 2021, *Crit Care Med* (Surviving Sepsis Campaign)	International guideline	Not applicable	No recommendation on restrictive vs liberal fluid after initial resuscitation (insufficient evidence)	Expert judgement where evidence is weak	Shows the uncertainty our pilot addresses

Feedback guide when reading drafts (not a grade)

Element	Good	Needs work
Search	String, database, date, filters and hits recorded; brackets correct	"We googled it"; no date; AND inside a block
References	5, relevant, include the best available design, exported to a reference manager	Websites, textbooks only, typed by hand, unread
Literature review worksheet	Same columns for every paper; limitation column filled	Copied abstract sentences
Problem statement	Known, then gap, then question and objectives; each fact cited	A literature dump; own data or results; unpublished local figures
Title & keywords	10–16 words; P, I/E, C and design visible; calibrated words; keywords add MeSH not in the title	"Fluids in the ICU"; abbreviations; "impact of" for an observational design
Integrity	Own words, every claim cited	Sentences pasted from abstracts

Exit question: answers

1. **OR** gives more hits.
2. Paper 1's primary outcome was **cumulative fluid balance at day 5 (ml/kg)**, not mortality.